6.1 Gastroesophageal Reflux Disease (GERD)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I have burning behind my chest after meals." • Worse when lying down **Objective** • Regurgitates food • Takes antacids daily	**ACTUAL** Acute pain related to reflux of gastric acid into the esophagus as evidenced by retrosternal burning after meals.	• Less heartburn within 1 week.	• Give the proton pump inhibitor before breakfast, as prescribed. • Advise small meals and nothing to eat for 2–3 hours before lying down. • Raise the head of the bed by 15–20 cm. • Suggest cutting out fatty food, coffee, smoking and tight clothing.	• Considered met as symptoms ease.
Subjective • "I did not know food affects this." **Objective** • Eats late at night • Drinks coffee often	**ACTUAL** Inadequate health knowledge related to lifestyle modification as evidenced by an unhealthy eating pattern.	• Names three lifestyle changes before discharge.	• Teach the dietary triggers and the value of weight loss if overweight. • Give written advice.	• Goal reached when the changes are listed.
Subjective *No specific complaint* **Objective** • Reflux at night • Nighttime cough	**RISK** Risk for aspiration as evidenced by risk factors: nocturnal reflux of gastric contents.	• No aspiration and no night cough.	• Keep the bed head raised by 15–20 cm. • No meals for 2–3 hours before lying down. • Give the prescribed acid-suppressing drug.	• Met if there is no cough or chest infection.

*References for this care plan: [32], [35], [37], [67]. Full entries are listed in the References section of the CPMN book.